



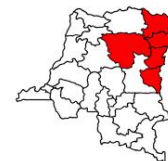
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°085/MVEBDB/07/08/2026

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HIGHLIGHTS (last 24 hours)

In the last 24 hours, 89 new confirmed cases (21 new deaths) of Ebola Virus Disease (EVD)

were recorded in the provinces of Ituri (73 cases), North Kivu (12 cases) and Haut-Uélé (4 cases), reflecting the continuation of active transmission, mainly in Ituri.

CUMUL CASES 4,209	ACCUMULATIVE DEATH CONFIRMED** 1916	LETHALITY 45.5%	PATIENTS IN ISOLATION / CTE 595	ACCUMULATIVE HEALED 828	TRACKING RATE CONTACTS (Of the Day) 83.4%
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Affected Provinces and Affected Health Zones

5 Provinces Affected
 Haut-Uélé, Ituri, North Kivu,
 South Kivu & Tshopo

53 Health Zones affected Out of 140 (37.8%)

246 Health Areas touched Out of 3104 (30%)

- In the past 24 hours, 89 new confirmed cases (21 new deaths) of Ebola Virus Disease (EVD) have been recorded in the provinces of Ituri (73 cases), North Kivu (12 cases) and Haut-Uélé (4 cases), reflecting the continuation of active transmission, mainly in Ituri.
- To date, the Democratic Republic of Congo has recorded 4,209 confirmed cases, including 1,916 deaths, representing a case fatality rate of 45.5%. Ituri remains the epicenter of the epidemic, accounting for 86.4% of cumulative confirmed cases and 82.0% of new cases reported in the last 24 hours.
- 18 patients were declared cured after obtaining two negative results at Ebola virus disease (EVD) (14 in Ituri and 4 in North Kivu) and 8 intra-CTE deaths recorded in Ituri.
- The epidemic affects 53 of the 140 health zones in the five affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23) and South Kivu (1/34), confirming the continued geographical spread of the disease.
- The National Coordination continues to strengthen surveillance, care, prevention and infection control interventions as well as community engagement in the most affected health zones in order to rapidly interrupt transmission chains.

Distribution of confirmed cases and deaths by affected province

(as of August 7, 2026)

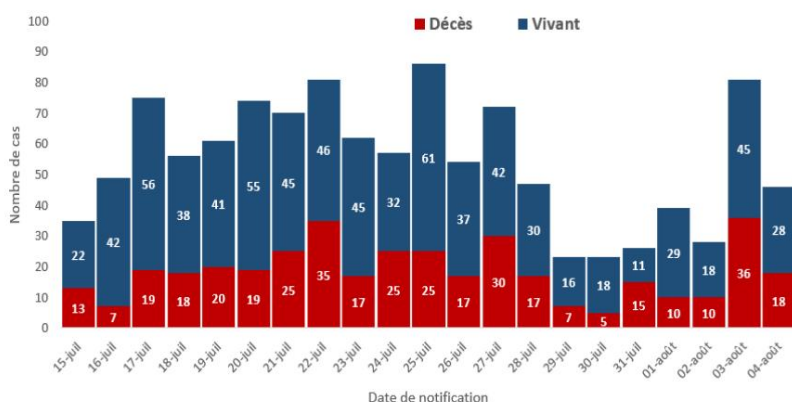
Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	3,636	1,551	42.7% 28/36	(77.8%) 67.0% 12/34	73
North Kivu	470	315	(35.3%) 48.4%	6/13 (46.1%)	12
Haut-Uélé	91	44	55.6% 6/23	(26.0%) 1/34 (2.9%)	4
Tshopo	9	5			0
South Kivu	3	1	33.3%		0

Total	4,209	1916	45.5% 53/140 (37.8%)	89
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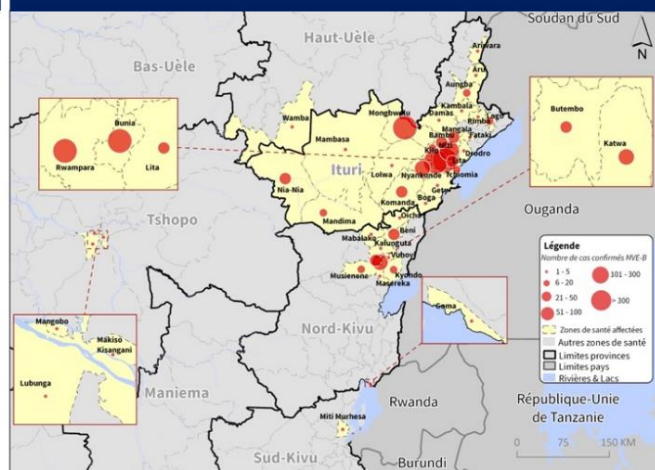
Status of alerts notified by ZS, as of August 7, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (as of today)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	13	0	13	12	0	1	0	10	10
Ituri	467	50	517	280	50	187	0	196	135
North Kivu	743	39	782	56	39	687	0	95	49
South Kivu	6	0	6	6	0	0	0	4	4
Tshopo	ND	ND	0	ND	ND	ND	ND	ND	ND
Total General	1229	89	1,318	354	89	875	0	305	198

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **Ituri** : Validation of data between the Surveillance, Laboratory, and Planning pillars and continued collection of data from health zones; strategic actions remain pending, notably the establishment of sub-coordination units in Wamba, Pawa, and Watsa.
- **North Kivu** : Support for the coordination of activities in the Musienene Health Zone; harmonization of alert and contact databases, and training of surveillance focal points.

1.1.2. Challenges

Persistent delay in operationalizing the zonal approach and insufficient completeness of data reporting, limiting local coordination, comprehensive data collection and rapid decision-making in the health zones concerned.

1.2. Epidemiological surveillance

1.2.1. Main Actions

1,318 alerts were recorded and verified in the four reporting provinces for August 7, 2026, of which 443 (33.6%) were validated as suspected cases and 305

(68.8%) were investigated and 198 suspects transferred to CT/CTE.

Contact tracing is at 83.4% (14,923 seen out of 17,896 to be followed), close to the 85% threshold, with weak performances in Haut-Uélé (50.1%) and Nord-Kivu (77.1%); Ituri reaches 88.8%.

1.2.2. Challenges

Poor performance of epidemiological surveillance, characterized by an investigation rate of 68.8% and suboptimal contact tracing, compromising early detection of cases and interruption of transmission chains.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

Fifteen alerts were notified to the PoE/PoC (9 in Haut-Uélé, 4 in Ituri, 2 in Nord-Kivu): The six alerts notified in Ituri and Nord-Kivu were validated, isolated and referred to the support structures, none of those in Haut-Uélé were.

148,287 travellers were checked (123,202 in Ituri, 25,085 in Haut-Uélé), with a screening rate of 98.2%.

Operationalization remains insufficient in Haut-Uélé (10/38; 26.3%); in Ituri, only one Strategic PoE/PoC on 18 (5.6%) operated without interruption, compared to 100% in North Kivu.

1.3.2. Challenges

Insufficient coverage and functionality of PoE/PoC, aggravated by the disruption of inputs, PPE and water in Haut-Uélé, the non-payment of service providers and the non-compliance with the health protocol by authorities at the PoC of Mukulia (North Kivu), limiting the effectiveness of surveillance at borders and along mobility routes.

1.4. Laboratory

1.4.1. Main actions

- **326 samples** were analyzed, confirming **89 new cases** of EVD (positivity rate: **27.3 %**).
- Ituri recorded **73 positive results, including 17 deaths**, while **North Kivu** and **Haut-Uélé** confirmed **12 (4 deaths)** and **4 cases respectively**.

1.4.2. Challenges

High proportion of positive samples in Ituri (26.1%) and incomplete reporting of laboratory results in North Kivu and Tshopo (no data transmitted), limiting biological surveillance and real-time monitoring of Ebola transmission.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- In **Ituri**, 11 rings were opened (46 in total), 32 sites identified and 31 decontaminated (97%); 56 EDS carried out on 74 death alerts (76%) and 44 bodies collected post-mortem, despite a completeness limited to 10 ZS out of 29 (34%).
- In **North Kivu**, 130 health facilities were supported and 453 providers were briefed; 9 case stay facilities were decontaminated, 42 EDS were carried out and 44 bodies were collected and secured, for a completeness of 5 out of 11 health zones. • **Tshopo** did not transmit any IPC/EDS data.

1.5.2. Challenges

Low implementation of Infection Prevention and Control (IPC) standards: very low reporting completeness in Ituri (34%) and North Kivu (5 health zones out of 11), partial decontamination of households of cases in Ituri (20 out of 66) and low card scores in North Kivu (29% in Katwa), demonstrating a continued risk of healthcare-associated infections.

1.6. Continuity of care

1.6.1. Main actions

1.6.2.

- **In Ituri**, 29 ambulances were assigned to the response, of which 25 were available; 43 trips were made, 42 of them successful, for 51 patients evacuated. The Ebola Treatment Centers (ETCs) recorded 85 admissions and 73 discharges, including 14 recoveries; occupancy reached 595 patients for 860 beds (69.2 %), with saturation of the Rwampara, ISTM and Nizi CTEs.
- **In North Kivu**, 247 cumulative admissions in CTE/CT, including 200 confirmed cases, for an intra-CTE mortality rate of 39%; 37 admissions, 4 recoveries and 8 deaths reported today.
- **In Haut-Uélé** : the absence of standardized CTE persists in the 6 affected health zones

1.6.3. Challenges

Insufficient availability of referral resources (4 ambulances out of service in Ituri, no ambulance in 7 health zones in North Kivu) and saturation of several CTE/CT facilities, limiting rapid access to care and isolation of suspects.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, continued interpersonal communications on the risks associated with caregivers and briefing of community ambassadors in the Mongbwalu health zone.
- **In North Kivu**, 20 motorcyclists were briefed and a guided tour of the CTE at HGR Beni was organized; 16 rumors were clarified, 1 community incident resolved and 103 household heads affected around 21 cases.
- **In Haut-Uélé**, 1,231 people reached during 351 home visits in Isiro, 30 motorcycle taxi drivers sensitized in Boma-Mangbetu and 55 IEC tools distributed (cumulative: 285).

1.7.2. Challenges

Persistent community challenges: demotivation of actors linked to non-payment for more than two months in North Kivu, denial of Ebola virus disease and opposition to a COVID-19 response in Isiro, repeated waiting on vaccine availability, likely to affect community support for response interventions.

1.8. Logistics

1.8.1. Main actions

In Ituri : the installed capacity reaches 840 beds (412 for confirmed cases, 428 for suspected cases); the expansion of the CTEs in Bambu, Nizi and Logo is requested in the face of saturation.

In North Kivu : continued unpacking of the WHO PCI kit donation, receipt of the order from the Kyondo Health Zone, delivery of PCI inputs to Oicha and 3 tanks of 3,000 litres to the PoE/PoC pillar; work in progress at the PoCs of Karuruma and Komba, at the CT of the Musienene General Hospital (MSF France) and at the isolation of the Vuhovi General Hospital (ULB).

1.8.2. Challenges

Persistence of logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation.

1.9. Security

1.9.1. Main actions

- In North Kivu, 6 security escorts were provided to EDS and PCI teams in Butembo and Oicha; major incident at the PoC in Mukulia (attack on service providers and security elements by provincial deputies) and equipment from the PoC in Vulindi was taken away.

1.9.2. Challenges

- Persistent insecurity, incidents involving political authorities and community resistance during dignified and safe burials, limiting access for response teams, continuity of interventions and coverage of activities in some affected areas: 6 dignified and safe burials not carried out in Ituri (Aru, Mangala and Mandima), 1 failure in North Kivu (Kitatumba CTE, Butembo) and 1 body taken by the family in Isiro, Haut-Uélé because of community resistance.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continuation of **briefing and awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA)** in the different provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse remains insufficient across all areas covered by the response.

KEY MESSAGES

- Ebola transmission remains active, with 89 new confirmed cases and 29 deaths (21 new and 8 intra Ebola treatment center) recorded in the last 24 hours, bringing the national total to 4,209 confirmed cases and 1,916 deaths.
- Ituri remains the epicenter of the epidemic, accounting for 86.5% of cumulative confirmed cases and 82.0% of new cases reported in the last 24 hours, with a more significant expansion (28 of the 36 health zones affected). • Surveillance remains suboptimal, with only 68.8% of alerts investigated, facilitating continued transmission. • Strengthening priority interventions (surveillance, care, infection prevention and control, logistics, and community engagement) remains essential to rapidly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



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